RISPANA SURGICAL & ENDOCRINE CENTRE
Endocrine Surgery | Head & Neck Surgery | Surgical Oncology
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Reg. No. UK/DDN/HOSP/2015/0094

OPERATION NOTES

Patient: Ms. Nandita Rawat Age / Sex: 34 / F IP No.: RSE/IP/26/0418

Date of surgery: 3rd March 2026

Surgeon: Dr. Manish Uniyal (Endocrine Surgery) Assistant: Dr. Kritika Bhatt

Anaesthetist: Dr. Rohit Semwal - GA with a nerve-monitoring (NIM) endotracheal tube

Pre-operative diagnosis: Papillary carcinoma thyroid (FNAC dated 14/02/2026: Bethesda VI); right lobe nodule 2.8 cm with suspicious central (level VI) nodes on USG.

Operation: Total thyroidectomy with bilateral central compartment (level VI) neck dissection

Findings

Right lobe: firm nodule 3 x 2.5 cm in the mid and lower pole, adherent to the sternothyroid, which was shaved off the capsule. Left lobe normal to palpation. Enlarged firm right paratracheal and prelaryngeal nodes, largest 1.2 cm. Both recurrent laryngeal nerves identified and preserved, with intact signals at the end of the procedure.

Steps

- Kocher collar incision two finger-breadths above the sternal notch; subplatysmal flaps raised.
- Superior pole vessels ligated close to the gland; both superior parathyroids preserved in situ.
- Right inferior parathyroid devascularised: minced and autotransplanted into the right sternocleidomastoid.
- Bilateral level VI clearance from the hyoid to the innominate artery.
- Haemostasis secured; no drain; skin closed with a subcuticular absorbable suture.

Specimens (thyroid with a stitch on the right superior pole; right and left level VI nodes in separate containers) sent for HPE.

Post-op orders: serum calcium and PTH at 6 hours and next morning; watch for tingling, stridor or voice change.

Dr. Manish Uniyal
M.S., M.Ch. (Endocrine Surgery)
Reg. No. UKMC-2287
Consultant Endocrine Surgeon

SYNTHETIC DOCUMENT — FOR DEMONSTRATION ONLY